MATERNAL-NEWBORN NURSING

FETAL POSITIONS & PRESENTATIONS

Recognize the picture.
Name the position.
Understand what it can mean for birth.

POSITION
Direction a fetal landmark points.

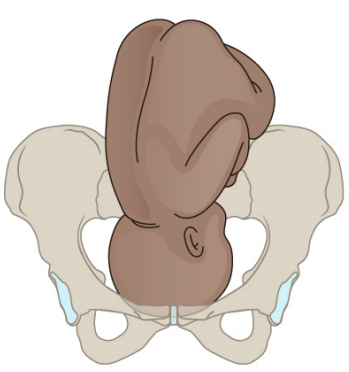
PRESENTATION
The part nearest the cervix.

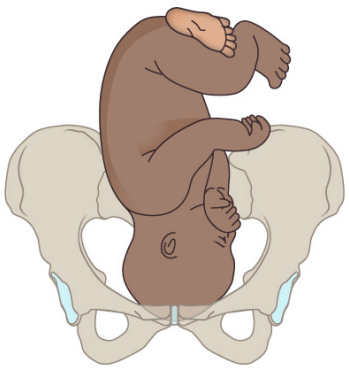
LIE
Baby's spine relative to mother's.

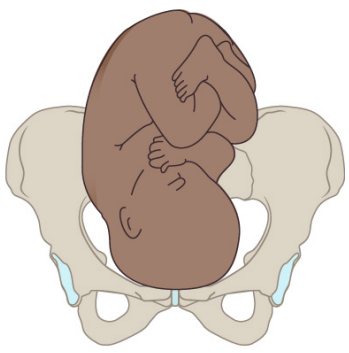
Read the letters: R/L = mother's right/left; O = occiput (back of head); M = mentum (chin); S = sacrum.
A = anterior (toward mother's front); P = posterior (toward her back); T = transverse (toward her side).

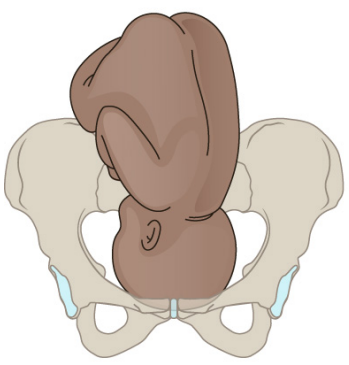
1 HEAD & FACE POSITIONS

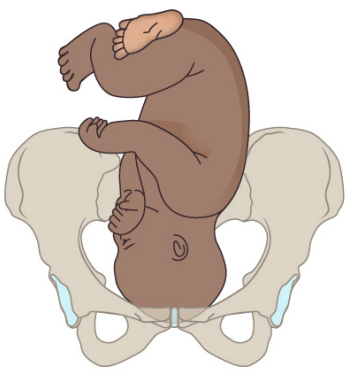
Cephalic = head first. Vertex: chin tucked; face presentation: neck extended.

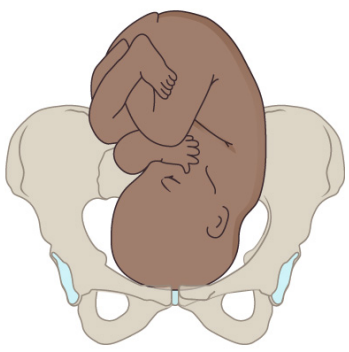

Right occiput anterior (ROA)

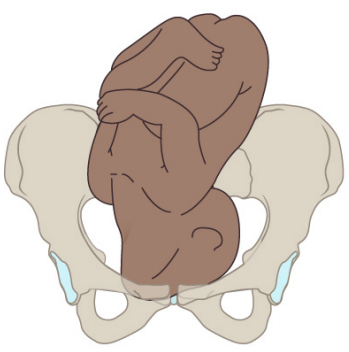

Right occiput transverse (ROT)

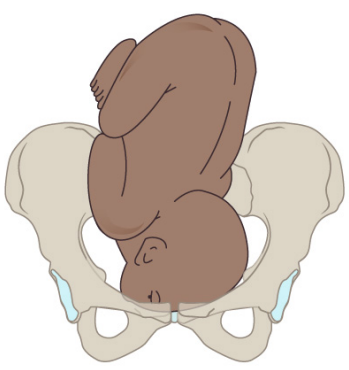

Right occiput posterior (ROP)

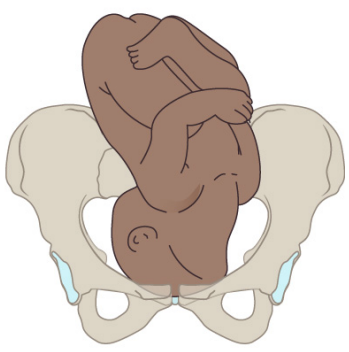

Left occiput anterior (LOA)


Left occiput transverse (LOT)


Left occiput posterior (LOP)


Right mentum anterior (RMA)


Right mentum posterior (RMP)


Left mentum anterior (LMA)

OCCIPUT ANTERIOR

ROA / LOA = right / left occiput anterior

Back of the head points toward mother's front; face points toward her spine.

Meaning: Usually favorable for vaginal birth.

OCCIPUT TRANSVERSE

ROT / LOT = right / left occiput transverse

Back of the head points toward mother's side.

Meaning: Often rotates during labor. Persistent OT can slow descent and require assistance.

OCCIPUT POSTERIOR

ROP / LOP = right / left occiput posterior

Back of head toward mother's back; face toward her abdomen ("sunny-side up").

Meaning: May cause back labor or a longer labor. Many rotate; persistent OP may need assisted or cesarean birth.

FACE PRESENTATION: CHIN MATTERS

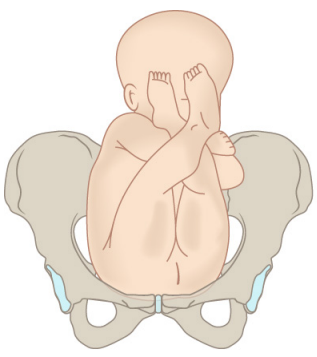
RMA / LMA: right / left mentum anterior. The chin points toward mother's front. **Vaginal birth may be possible.**

RMP: right mentum posterior. The chin points toward mother's back. **Persistent chin-posterior face presentation requires cesarean birth.**

Nursing: Confirm by examination/ultrasound; monitor fetal heart rate and labor progress. Position can change; the name alone does not decide delivery mode.

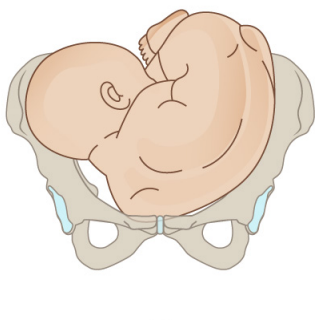
2 BREECH & SHOULDER PRESENTATIONS

The letters (a-d) match the four panels in your picture.


(a)


(b)


(c)


(d)

a FRANK BREECH

Buttocks first; hips bent, knees straight, feet up.

Meaning: Breech birth planning is needed.

b COMPLETE BREECH

Buttocks first; hips AND knees bent.

Meaning: Breech birth planning is needed.

c FOOTLING BREECH

One or both feet lead.

Meaning: Higher cord-prolapse risk; cesarean is generally planned at term.

d SHOULDER / TRANSVERSE

The whole baby lies sideways; a shoulder may lead.

Meaning: Persistent transverse lie in a term singleton usually requires cesarean.

Also know: Oblique lie = diagonal; reassess near birth. **Brow presentation** = partly extended neck, forehead first; it may convert to vertex or face. Persistent brow at term usually needs cesarean.

Breech at term: Selected patients may have ECV (external cephalic version) around 36-37 weeks. Most persistent breech babies have planned cesarean birth; vaginal breech birth requires careful selection and an experienced team.

Sources: ACOG: breech | ACOG: labor | RCOG | Merck | SASGOG | Face/brow | Updated October 18, 2026